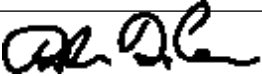


Guideline Title: Multi-System Trauma – Adult & Pediatric

Guideline Number: GUID-3203-T008

Issue date: 06/06/2016	Replaces Dept. Policy: GUID-3203-5016
Revision dates: 07/01/2026	Developed by: Air Care Team
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Signature: 	Signature:
Department Numbers: 3203	

- I. **PURPOSE:** To provide guidelines for initial trauma resuscitation and timely application of emergency measures to minimize the morbidity and mortality of traumatic injuries. The medical transport team must consider the capability of the receiving facility to manage pediatric trauma patients.
- II. **DEFINITIONS:**
- III. **DEPARTMENT GUIDELINE:**
 - a. Assess scene for hazards.
 - i. Assist as requested in caring for patients during extrication, being certain not to jeopardize their own safety at any time.
 - ii. Note damage to vehicle. If time permits (i.e., extrication in progress), document mechanism of injury (using photographs if available).
 - b. Assess patient(s). Perform primary survey, treating all immediately life-threatening findings.
 - c. **Interventions**
 - i. Control life threatening external hemorrhage using well-aimed direct pressure/pressure dressings, with tourniquets per *GUID3203-PR006 Tourniquet Therapy*, or with Quik Clot gauze per *GUID3203-PR028 - Hemostatic Dressing – QuickClot Gauze*.
 - ii. If patient has adequate spontaneous respirations, administer supplemental oxygen to maintain SpO₂ > 93%.
 - iii. If patient has a Glasgow coma score (GCS) of < 8 or has respiratory failure, advanced airway management should be considered per *GUID3203-G008 – Rapid Sequence Intubation for Endotracheal Intubation* and *GUID3203-PR020 – Endotracheal Intubation*. If laryngoscopy or passage of an ET tube is not possible follow *GUID3203-PR016 – King LTS-D Laryngeal Tube* or *GUID3203-PR007 Cricothyrotomy - Surgical*.
 - iv. All patients at risk for spinal injury need to be immobilized.
 1. Apply padding to correct for larger occiput in smaller children under 2 years old. Utilize pediatric immobilization device when possible. Younger children may be secured in their car seat if car seat appears intact. Hold manual c-spine until pediatric cervical collar is applied and secure the head with rolled towels to restrict further motion.

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2. For interfacility transports where the immobilization devices have been removed, review radiology exams completed and then consult with sending or receiving physician regarding selective spinal motion restriction.
 - a. Suspected cervical spine injuries should always remain in a c-collar unless body habitus or clinical situation does not allow it.
 - b. Suspected thoracic or lumbar spine injuries do not require a long back board but transfer to and from stretchers should always be performed with multiple staff members and a slide board to maintain spinal precautions.
- v. Obtain adequate vascular access per *GUID3203-PR019 Intravenous Insertion - Peripheral*.
 1. Preferably obtain 2 large bore IV's (16 gauge for adults, 20-18 gauge for children).
 2. If rapid vascular access cannot be gained, intraosseous (IO) should be considered per *GUID3203-PR013 Intraosseous Infusions*.
- vi. Continuous monitoring of cardiac ECG, pulse oximetry, non-invasive blood pressure, Glasgow coma scale, and Revised Trauma Score.
- vii. If patient displays signs of inadequate perfusion, begin resuscitation per *GUID3203-M011 – Hypotension* or *GUID3203-PED009 – Pediatric Hypotension*. Limit IV fluids in trauma patients. If active or suspected hemorrhage, treat with blood products per *GUID3203-PR025 – Emergency Blood Product Administration*. For burn patients treat per *GUID3203-T002 – Burns*.
 1. For adults, utilize pressure bag for rapid infusion if indicated.
 2. If the source of bleeding is not obvious and mechanism (major MVC/motorcycle/ATV/fall from >30 feet/blast injury) or physical exam (unstable pelvis) is suspicious for open book pelvic fracture place a pelvic binder per *GUID3203-PR027 Pelvic Circumferential Compression Binder*.
 3. Utilize fluid warmer when possible, per *GUID3203-PR026 Warming of Blood Products & Intravenous Fluids*.
 4. Pediatrics: May administer volume rapid IV push via syringe for very small patients
- viii. For extremity injuries, treat per *GUID3203-T009 Musculoskeletal Trauma*
- ix. Stabilize and do not remove penetrating objects from wounds. If transport cannot safely be accomplished in an aircraft due to penetrating object, alternate mode of transport must be initiated.
- x. Apply saline dressings to any eviscerated bowel or dry dressings to any open wounds.
- xi. Splint/immobilize extremity fractures. Assess traction splint. Continually monitor distal pulses, motor, and sensation to the site of injury.
- xii. Do not place nasogastric tubes in patients with significant facial trauma.
- xiii. Treat nausea/vomiting per *GUID3203-G004 - Motion Sickness/Nausea*.
- xiv. Treat pain and/or anxiety per *GUID3203-G005 - Analgesia and Sedation Management*.
- xv. Contact medical control if patient entrapped and field amputation is considered lifesaving.
- d. If abuse is suspected: For pediatric patients suffering traumatic injury, the provider must consider physical abuse as part of the differential diagnosis. The State of Florida requires medical and pre-

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hospital personnel to report instances of suspected abuse to local child protection agencies. It may be required that each mandated individual generate a report after contacting a suspected abuse victim. The transport team must obtain a careful history to compare reported mechanism of injury with observed or diagnosed injury patterns. Inconsistency here or the “story not matching up,” should lead the team to suspect potential abuse. This should be reported not only to local child protection entities, but also to the receiving facility.

IV. **DOCUMENTATION:** EMS Charts

V. **REFERENCES:**